ONE Clinical Behavior Management

POSSIBLE F-TAG/QMS

Resident Rights

- Self-Determination

Quality of Life

- ADL Maintenance
- Activities meet Interest/needs

Behavioral Health

- Sufficient/Competent Staff-Behavioral Health Needs
- Treatment/Svc for Mental/Psychosocial
- No Pattern of Behavioral Difficulties Unless Unavoidable
- Treatment/Service for Dementia

Pharmacy

- Drug Regimen is Free from Unnecessary Drugs
- Free from Unnecessary Psychotropic Meds/PRN Use

THERAPIES

- PT
- OT
- SLP

PURPOSE OF THERAPY

- Help our residents with dementia, cognitive impairments, and unwanted behaviors to be as safe and independent as possible with utilization of sensory strategies, activity interest/engagement, and adapted care approaches

GOAL EXAMPLES

PT:

- Patient will safely perform bed mobility tasks with Mod (A) and 50% Verbal Cues and 50% Tactile Cues for initiation/termination of tasks and for proper sequencing in order to get in/out of bed.
- Patient will be able to perform functional transfers with Mod A from caregivers utilizing most appropriate technique and assistive device.
- Patient will perform supine to sit transfers with moderate assistance with 100% multi-sensory cues and less resistance to care.

OT:

- Patient will perform grooming and hygiene tasks with moderate (35-50%) verbal, visual, tactile cues for correct sequencing tasks provided by trained caregivers.
- Patient will decrease negative behaviors during morning care on 4/5 sessions.
- Patient reports falling back to sleep within 20 minutes if waking during night 5/5 sessions.
- Patient reports sleeping 7 to 8 hours per night 5/5 sessions.

TOOLS & ASSESSMENTS

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- ACLS-5
- FAST/GDS
- Cognitive Assessment tool guide
- SLUMS/MOCA
- PAINAD
- FLCI
- ABCD
- Activity Card Sort
- RTI
- Routine Task Inventory
- Sensory Profile
- Family/Staff interviews
- Life History Profile

IMPLEMENTATION:

- Identify strategies to assist with recall of environmental information (aides, Spaced Retrieval techniques, etc.).
- Identify effective cueing strategies to increase processing of and attention to task demands
- Identify strategies to increase patient safety within current environment
- Identify stimuli that elicits a desired communicative response
- Integrate sensory strategies into daily routines to enhance meaningful engagement with patient

POSSIBLE F-TAG/QMS	THERAPIES	PURPOSE OF THERAPY	GOAL EXAMPLES	TOOLS & ASSESSMENTS
Quality Measures • Antipsychotic Medications/Behavior Symptoms Affecting others • Antianxiety/Hypnotic/Depression Symptoms • Falls with Major Injury • Claims Based: ER Visits Hospitalizations			• Patient will engage in meaningful activity for 10 minutes with therapist to redirect unwanted behavior for 3/3 sessions. • Patient will engage in meaningful activity for 10 minutes with trained caregiver to redirect unwanted behavior for 3/3 sessions. • Resident will reduce pain to 3/10 with use of modalities prior to ROM to prevent contractures. **ST:** • Patient will engage in simple verbal communication using life story board for 3 minutes prior to engaging in ADLs in order to decrease refusal agitation with moderate verbal and visual cues. • Patient will sequence 5/5 steps to place a call to family member on cell phone, with use of visual sequence aid and moderate vanishing cues, to increase ability to make phone calls to family members to communicate daily wants/needs. • Patient will use communication supports in a field of 4 to indicated 3 basic needs to caregivers on 4/5 opportunities in order to communicate basic needs. • Patient will state and demonstrate 3 word finding strategies during therapy session in order to improve communication of needs and decrease frustration during communication.	• Use of environmental modifications for lighting, sounds such as white noise machines, or other equipment that can increase or decrease sensory stimulation a space provides • Use of remedial interventions such as activities that encourage engagement of tactile, proprioceptive, and movement opportunities and promote motor planning, such as cooking or meal prepping activities • Use of sensory diet programs that are individualized such as rocking chair, quiet space, aromatherapy, weighted blanket, stress balls, fidget items, etc. to help manage sensory modulation problems or reduce symptoms of anxiety or agitation to encourage participation in needed or desired activities • Identify optimal ADL set up to facilitate function and reduce excess disability • Modify task demands to match the cognitive level of the patient • Optimize set-up and environment • Train facility staff on ACL levels and individualized approaches for residents • Provide ADL and activities prescriptions/guides for residents to facility staff • Heart rate variability and strategies training to patients and staff • Sleep hygiene training to patients and staff • Pain management through modalities, Dry Needling, Cupping, use of communication supports and visual aids to communicate pain effectively

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			OT/ST: • Resident will utilize call button 100% of the time in room with use of visual cues to decrease risk for falls. • Pt will participate in activity for 5 minutes with therapist 3/3 sessions to reduce agitation and facilitate improved quality of resident care. • Pt will participate in activity for 5 minutes with trained caregiver for 3/3 sessions to reduce agitation and facilitate improve quality of resident care. • Resident will engage in leisure task of choice with min (hand over and/tactile/verbal/visual) cues to facilitate engagement . • Pt will follow visual schedule with min verbal cues to facilitate participation in facility activities. • Pt will demonstrate orientation/navigation in the facility using predetermined visual cues with less than 25% verbal cues. • Pt will follow (1, 2, 3,4) step commands with 75% accuracy to improve ability to engage in group activities. • Pt will identify 2 out of 4 hazards in a given situation to improve safety awareness in the facility. • Pt will demonstrate functional problem solving during familiar situations with 50% visual/verbal cues in order to improve decision making skills.	

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			Pt will improve sustained attention to greater than 10 minutes to facilitate participation in leisure tasks. Pt will participate in 1 group activity of choice per week to improve social engagement and reduce social isolation. Pt will complete structured job task 2 days per week with set up and less than 2 errors to improve social engagement and self-confidence. Pt will decrease signs of agitation during daily routine as evidence by caregiver report of less than 4 outbursts per shift. **PT/OT/ST:** Pt will participate in HRV tasks utilizing biofeedback with 100% participation provided consistent verbal cues/redirections for reduced heart rate, stress, and anxious behaviors. Pt and staff will utilize strategies developed during HRV training with during 100% of opportunities provided direct training/education by SLP in order to decrease negative behaviors with peers/caregivers during time of increased stress and/or anxiety within current living environment. Pt will demonstrate ability to maintain 35% "High" coherence during 1 meaningful activity utilizing specific breathing/calming strategies in order to promote engagement in activities, comfort and overall quality of life.	